

HOW TO OPEN A GROUP HOME BUSINESS in Florida

Foster homes & group homes under the iBudget waiver — serving adults with IDD.

A complete, plain-language, step-by-step startup guide — written so you can actually DO each step.

Figures current for 2026 · Every contact, rate & fee should be re-verified before you rely on it.

The Florida model — read this first

“Group home” in Florida means providing a real home to adults with intellectual or developmental disabilities (IDD) and delivering Residential Habilitation — everyday support, supervision, and skill-building — instead of an institution. The state funds it through the iBudget Florida waiver, a Medicaid home- and community-based services (HCBS) waiver run by the Agency for Persons with Disabilities (APD). Two things make Florida distinctive, and they run through everything below. First, it's a LICENSED-FACILITY state: even a small home for three people is a licensed residential facility under Rule 65G-2, F.A.C. — inspected before licensure, fire-inspected annually, monitored by APD monthly, and running monthly fire drills. Second, there are TWO SIZES with one licensing backbone: a foster care facility (up to three residents — the small, host-home-style start) and a group home facility (four to fifteen residents — the bigger operation), both licensed under 65G-2 and both funded by iBudget.

The two ways in. *Path A — open a licensed FOSTER HOME (up to 3 residents). The smallest, fastest, lowest-cost entry, closest to a host home; you can often run it yourself with relief help, and a special “Live-In” rate is available for homes licensed for three or fewer. Path B — open a GROUP HOME (4–15 residents). A bigger operation with paid staff, more fire-code requirements, and more revenue. Many owners open a foster home first, learn the ropes, then grow into one or more group homes. Either way the licensing backbone is the same three tracks — enroll as an APD provider, license the facility under 65G-2, and get a “Full” Medicaid provider number from AHCA — and this guide walks all three.*

The names you'll deal with. *APD = Agency for Persons with Disabilities (runs the iBudget waiver, enrolls providers, and licenses and monitors your home). iBudget Florida = the Medicaid HCBS waiver that funds services — each client has an individual budget (their “iBudget”); it's fee-for-service, not managed care. AHCA = the Agency for Health Care Administration, Florida's Medicaid agency — you get your Medicaid provider number here. WSC = Waiver Support Coordinator, the person who writes each client's support plan and cost plan and coordinates services (Florida's case manager). QSI = the Questionnaire for Situational Information, the assessment that helps set each client's needs and budget.*

How to use this guide

Read it once start to finish, then use it as a checklist and a phone list. It's written for someone who has never opened a home — every time we name a thing, we stop, break it down, and tell you exactly where to go and what to ask. It is general information, not legal, tax, or billing advice; requirements, rates, and codes change, so verify each step with APD, AHCA, your local fire authority, and qualified local counsel (see Part 12) before you rely on it.

The journey at a glance

These milestones take you from idea to your first payment — whether you start with a small foster home or a full group home.

1	Decide your path & set up the business	Path A (foster home ≤3) or Path B (group home 4–15); entity, EIN, NPI, insurance (Parts 3–4)
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2	Learn the standards & train	Read 65G-2 and the iBudget Handbook; complete APD Pre-Service Training (Part 5)
3	Pass Level 2 background screening	You and required household members / staff (Part 5)
4	Enroll as an APD provider	The iBudget provider application for Residential Habilitation — it's free (Part 5)
5	License the facility (65G-2)	APD licensing application, facility inspection & HCBS-settings survey (Parts 5–6)
6	Get your “Full” AHCA Medicaid number	Enroll to bill and be paid directly by Medicaid (Part 5)
7	Staff & write your policies	Direct service providers, required training, documentation (Parts 7–8)
8	Get a resident placed	APD eligibility, iBudget, support plan, WSC match (Part 9)
9	Deliver services & get paid	Bill Residential Habilitation in FMMIS per day (Part 10)

Rule of thumb. Start smaller than you think. A licensed foster home for three is faster, cheaper, and simpler to stand up than a full group home — fewer fire requirements, a lighter staffing plan, and less overhead — and it lets you learn APD's system, the 65G-2 standards, and the WSC relationships on a small scale. Many of Florida's strongest operators opened a foster home first and grew into group homes once they knew the terrain. The licensing backbone is identical, so nothing you learn is wasted when you scale up.

Before you begin — a realistic picture

Two honest truths will save you time and money. First, this is a relationships-and-documentation business funded by Medicaid, not a real-estate play or a normal small business: your revenue depends on getting your provider enrollment, your 65G-2 license, and your AHCA Medicaid number all in place, and then on WSCs choosing your home for the people they serve — so your reputation with APD, your regional licensure specialist, and the WSCs in your area is your most valuable asset. Second, the money arrives on a delay and only after a resident is placed: provider enrollment, licensing (with an inspection and an HCBS-settings survey), and the AHCA number take several months end to end, and you don't bill until a resident's support plan authorizes residential habilitation in your home — so you need patience and a working-capital cushion to carry setup and any staff costs through the ramp. Start with a foster home if you can, learn the system on a small scale, and grow into a group home once you know the terrain.

Here's how the rest of this guide is organized: Parts 1–2 explain what the business is and who regulates and pays you; Part 3 sets up the business itself, with the EIN, NPI, bank account, and insurance broken all the way down; Part 4 walks the two paths in; Part 5 is the enroll-license-enroll chain that unlocks payment; Part 6 covers the home, the 65G-2 checklist, and fire safety; Parts 7–8 are staffing and the policies and records APD requires; Part 9 is how a person actually gets placed with you; Part 10 is how you get paid, with the real billing codes and the money math; Part 11 covers costs, timeline, and getting placements; and Part 12 collects every phone number and script in one place, with Miami and Tampa worked out. Read it once end to end, then keep it beside you as a checklist.

PART 1

What This Business Is

In short: *Providing a licensed home and Residential Habilitation to adults with intellectual or developmental disabilities under the iBudget waiver — in a foster home (up to 3) or a group home (4–15). Funded by Medicaid, run by APD, paid after you're enrolled, licensed, and delivering services.*

A “group home” business in Florida is really a developmental-disabilities support business. You help an adult with an intellectual or developmental disability live successfully in the community — with daily support, supervision, skill-building, help with health and medications, and a real home life — rather than in an institution. Florida's core residential waiver service for this is Residential Habilitation: supervision and training in daily-living, homemaking, and social skills, delivered in a home you've licensed under APD. That home comes in two sizes — a foster care facility for up to three residents, or a group home facility for four to fifteen — and both run on the same iBudget waiver.

Be clear-eyed about what kind of business this is. This is a human-services business built on relationships and documentation, funded by Medicaid — not a real-estate play. Your value is the quality and reliability of the home and the support you provide, and your ability to keep it compliant, licensed, and well-documented, because that's what APD licenses and monitors, what the iBudget cost plan authorizes and pays for, and what keeps people safe. Go in understanding that the care and the paperwork ARE the business.

Why this business — and why now

Demand for community-based homes for people with developmental disabilities is high and durable: Florida, like every state, is moving people out of institutions and into the community, families overwhelmingly want a real home for their loved one, and the iBudget waiver funds exactly this. Florida has a large waiting list for waiver services, so the need for good licensed homes is real and ongoing. Because WSCs and families choose the home that fits each person, a reliable, well-run, HCBS-compliant home that builds relationships with WSCs can keep its beds full. And you can start lean — a licensed foster home for three that you can often run yourself with relief help — and grow into group homes as you learn the system.

Be honest about the two realities that trip up new operators. (1) *It's a licensed facility, not a casual arrangement: even a three-person foster home is inspected, fire-inspected annually, monitored monthly by APD, and runs monthly fire drills — plan for real recordkeeping from day one.* (2) *Room and board is separate from your pay: Medicaid pays for the SERVICE (residential habilitation), not the resident's rent and food, which come from their own income (usually SSI) under a residency agreement. Plan your business — and explain it to families — around both.*

PART 2

Who Regulates You & Who Pays You

In short: APD enrolls you, licenses your home under 65G-2, and monitors you; AHCA gives you the Medicaid provider number that lets you be paid; and each resident's iBudget cost plan — built by their WSC — authorizes the service. Learn who does what, because the pieces have to line up before a claim pays.

APD — Agency for Persons with Disabilities (your enroller, licenser & monitor)

APD runs the iBudget waiver, enrolls you as a provider of Residential Habilitation, licenses your foster or group home under Rule 65G-2, and monitors it (at least monthly). Without APD provider enrollment and a 65G-2 license you cannot deliver these services. APD works through regional offices, and your regional licensure specialist is the single most important person in your licensing — they review your application, conduct the facility inspection, and run the HCBS-settings survey. APD's statewide line is 1-866-273-2273 (1-866-APD-CARES), and provider and licensing information lives at apd.myflorida.com.

AHCA — Agency for Health Care Administration (your Medicaid payer)

AHCA is Florida's Medicaid agency, and it issues your Medicaid provider number. To be paid by APD you must enroll with AHCA as a “Full” provider — the option that lets you bill and receive payment directly from Medicaid. A “limited” enrollment only lets managed-care plans pay you, and the enrollment type can't be changed later without starting over — so choosing “Full” is one of the most important, easiest-to-get-wrong steps in the whole process. You submit claims through FMMIS, the Florida Medicaid Management Information System.

The WSC & iBudget — how each person's service is authorized

Because iBudget is fee-for-service (not managed care), there's no health plan between you and Medicaid — but each resident's service still has to be authorized on their cost plan. The Waiver Support Coordinator (WSC) is Florida's case manager: they develop each client's support plan and cost plan, informed by the QSI assessment, identify residential habilitation as a needed service, and — with the person's and family's choice — match them to a licensed home like yours. The WSC network is where your referrals come from, so those relationships are your census.

What this means for you: the path to payment is a chain — APD (enroll and license) → AHCA (get your Full Medicaid number) → a WSC authorizing residential habilitation on a resident's cost plan → you deliver and bill in FMMIS — and each link takes time. Start them in the right order and in parallel where you can, and treat your APD regional office and the WSCs as key relationships. A home that's enrolled, licensed, Full-enrolled with AHCA, and known to WSCs is a home that fills.

PART 3

Form Your Business & Insure — Step by Step

In short: *This is where most first-timers get stuck. We break each piece all the way down — what it is, where to go, exactly what to do, and what to have in hand.*

3.1 Form your legal entity (an LLC)

An “entity” is the legal business — almost always a Limited Liability Company (LLC) — that holds your license or registration, your contracts, and your bank account, and that shields your personal assets if the business is ever sued. In Florida you form the LLC through the Division of Corporations (Sunbiz), part of the Department of State. Your entity, EIN, and NPI all have to be in place and consistent before you enroll with APD, license the facility, and enroll with AHCA, because they're checked along the way. You'll name a “registered agent” (a person or service with a physical in-state address who receives legal mail — this can be you) and adopt an Operating Agreement (the internal document that sets out who owns what and who can make decisions).

- 1. File your Articles of Organization with Sunbiz.** Online at sunbiz.org — the Florida LLC filing fee is \$125 (that's a \$100 filing fee plus a \$25 registered-agent designation). Name the LLC and list a registered agent with a Florida street address (this can be you).
- 2. Adopt an Operating Agreement.** The internal document that sets ownership and management — your bank, your APD provider file, your 65G-2 license, and your AHCA enrollment will all reference your structure.
- 3. File your Annual Report every year.** Florida LLCs must file an Annual Report with Sunbiz between January 1 and May 1 each year to stay active — the fee is \$138.75, and a late report carries a steep \$400 penalty, so calendar it.
- 4. Confirm local business tax receipts.** Many Florida cities and counties require a local Business Tax Receipt (formerly an occupational license) — check what applies where you'll operate.

No newspaper publication in Florida. *Unlike a few states, Florida has NO LLC publication requirement — forming the entity is a straightforward Sunbiz filing plus the annual report. Don't pay a service that tells you otherwise.*

3.2 Get your EIN — the free federal tax ID

An EIN (Employer Identification Number) is your business's federal tax number — think of it as a Social Security number for the company. It's free, takes about ten minutes, and you need it before you can open a bank account, run payroll, or enroll with any payer. Here's exactly how to get it:

- 1. Go to IRS.gov and open the EIN Assistant.** Search “Apply for an EIN online.” Apply directly on the IRS website — never pay a third-party site that charges for this free service.
- 2. Choose your entity and enter the responsible party.** Select “Limited Liability Company,” then give the responsible party's legal name and SSN or ITIN (usually you, the owner), plus your business name and address.
- 3. Finish in one sitting.** The online session times out after about 15 minutes of inactivity and can't be saved partway — have your details ready before you start.
- 4. Save your EIN and the CP-575 letter.** You get the EIN on the spot and can download the official CP-575 confirmation letter — save both; your bank and every payer will ask for them.

3.3 Get your NPI — the national provider ID

An NPI (National Provider Identifier) is a free, ten-digit ID number that health-care payers use to identify your organization. You'll use it to enroll with AHCA as a Full Florida Medicaid provider and to bill residential habilitation in FMMIS. Your AGENCY needs a Type 2 (Organizational) NPI — which is different from the Type 1 (individual) NPI a single person would get.

- 1. Create an I&A account.** At nppes.cms.hhs.gov, set up an Identity & Access (I&A) account for whoever will manage the number — usually an owner.

2. Start a Type 2 (Organizational) application. Log into NPPES and choose Type 2, not Type 1. Type 1 is for an individual person; your business is a Type 2 organization.

3. Enter your legal name, EIN, and address, and pick your taxonomy. Choose the taxonomy that fits a developmental-disabilities / community-based residential (group home) provider — confirm the exact one AHCA/APD want. The “taxonomy” is the code that says what kind of provider you are — confirm the exact one your Medicaid program wants before you submit.

4. Submit — it's free and quick. It usually processes in about 10 business days. Save the NPI; you'll use it on every enrollment and claim.

3.4 Open a business bank account

Keep the business's money completely separate from your personal money — it protects your liability shield and makes payroll, taxes, and payer deposits clean. Open the account once your entity and EIN are set up.

What to bring to the bank:

- Your EIN confirmation (CP-575) from the IRS
- Your filed Articles of Organization / Certificate of Formation and the entity ID number
- Your signed Operating Agreement
- A Beneficial Ownership form (the bank collects details on anyone owning 25%+ plus a control person)
- A government photo ID, SSN, date of birth, and home address for each owner/signer
- An opening deposit (often around \$100)

Where to go: a bank with lots of Florida branches — Bank of America, Truist, Wells Fargo, Regions, Chase, or a strong local/community bank or credit union (Suncoast Credit Union and VyStar are large Florida credit unions). Home care is payroll-heavy — you pay caregivers weekly, but Medicaid and insurers pay you weeks later — so ask specifically about a business line of credit or invoice financing to bridge payroll before you scale.

3.5 Get your insurance — what you need, who sells it, and what to ask

A licensed residential facility houses and supports vulnerable adults around the clock, so you need coverage built for a human-services / developmental-disabilities provider — and APD, your fire authority, and your lender will expect proof:

Coverage	What it protects against	Must-have?
General & Professional Liability	Injury or property damage, plus claims about the CARE and supervision you provide	Yes — foundational
Abuse & Molestation	Allegations of abuse of a resident — general liability EXCLUDES this	Yes — high exposure
Property / Facility	Fire, storm (Florida hurricane exposure), and damage to the licensed home	Yes — you house people
Directors & Officers / Management	Regulatory, employment, and governance claims	Recommended
Non-Owned & Hired Auto	Staff driving their own cars to transport residents to appointments	Yes — critical
Workers' Compensation	Staff injuries	Required in FL (4+ employees)

Abuse & molestation is your highest exposure. Housing and supporting vulnerable adults means abuse & molestation coverage — EXCLUDED by standard general liability — is essential, with a real limit, not a token sub-limit. Confirm it explicitly. And because you own or lease a Florida home, confirm your property policy handles hurricane/wind and flood the way Florida requires.

Real places to get it

Buy through an independent agent or broker who writes HUMAN-SERVICES / developmental-disabilities provider programs — this is a specialty niche. Carriers and programs active in DD/IDD residential services include NSM/Irwin Siegel Agency (a social-services specialist), Negley Associates, Philadelphia Insurance (PHLY), and CNA. Find a local Florida agent at [trustedchoice.com](https://www.trustedchoice.com) and ask specifically for a developmental-disabilities / group-home / APD-provider program. Confirm the carrier writes this in Florida before you rely on a quote.

What to ask the insurance agent

“Do you write developmental-disabilities / APD group-home & foster-home providers in Florida — which carriers, and what’s the minimum premium?”

“Is abuse & molestation included, at what limit, and is it a sub-limit or a full limit?”

“How does the property policy handle Florida hurricane/wind and flood on the licensed home?”

“Do you include non-owned & hired auto for staff transporting residents?”

“Can you set up Florida workers’ comp for my direct service staff?”

3.6 Keep your paperwork consistent — every step downstream checks it

One practical warning that saves Florida operators real time: your legal name, ownership, EIN, and NPI have to line up EXACTLY across every step that follows — your APD provider enrollment, your 65G-2 licensing application, and your AHCA Medicaid enrollment. Your information passes through multiple state systems before a claim pays, and a mismatch at any handoff — a slightly different business name here, an old address there, a Type 1 NPI where a Type 2 belongs — is one of the most common and most frustrating causes of a stalled enrollment or a rejected claim. Set the entity up once, correctly, and make every downstream document point at the same clean legal picture. It’s worth an hour with your accountant or attorney up front to confirm the structure, because untangling a mismatch after you’ve applied costs far more than getting it consistent now. And keep your Sunbiz annual report, your APD license renewal, and your AHCA enrollment calendared — letting any one lapse stops your payments even when the care is going perfectly.

The business foundation to have in hand before you apply to APD:

- Your formed Florida LLC (Articles of Organization filed on Sunbiz, annual report current) and Operating Agreement
- Your EIN confirmation (CP-575) from the IRS
- Your Type 2 (organizational) NPI from NPPES
- A business bank account opened in the entity’s exact legal name
- Bound insurance — general/professional liability, abuse & molestation, property (with hurricane/flood), non-owned auto, and workers’ comp
- A single, consistent legal name, ownership list, and address used identically on every document

With those six things in place and consistent, you’re ready to build the enrollment and licensing applications in Part 5 on a clean foundation — and you’ve removed the paperwork mismatches that stall more Florida applications than any substantive problem does.

PART 4

The Two Paths — Foster Home (A) or Group Home (B)

In short: Path A is the fast, small way in — a licensed foster home for up to three residents you can often run yourself. Path B is the bigger business — a group home for four to fifteen, with paid staff and more fire code. Same licensing backbone; pick the one that fits where you are today. Many start with A and grow into B.

4.1 Path A — open a licensed foster home (≤3)

A foster home means up to three adults with IDD live in your home and you support them through everyday life — meals, routines, appointments, the good days and the hard days. It's a licensed facility under 65G-2, so you're taking on real recordkeeping and inspections, but it's the fastest, cheapest way in, and the closest thing to a host home. Everyone in the household who's 18 or older will be background-screened, so it's a family decision.

- 1. Make sure it fits your life.** Everyone in the household is part of this, and adults in the home get screened — so it's a family decision. Go in with eyes open about the good days and the hard ones.
- 2. Secure the home and complete Pre-Service Training.** APD requires the home secured and available at the time of application, and you must complete the required Pre-Service Training before you submit your iBudget provider application. Check APD's site for training dates.
- 3. Pass Level 2 background screening.** You and required household members complete fingerprint-based Level 2 screening (ss. 393.0655, 435.04, 409.907, F.S.). You can't be suspended or terminated as a Medicaid/Medicare provider anywhere.
- 4. Enroll, license, and get your Full AHCA number.** Enroll as an APD provider for Residential Habilitation, license the foster home under 65G-2 (with the inspection and HCBS-settings survey), and get your “Full” AHCA Medicaid number — the same three tracks in Part 5, at the small scale.

The Live-In rate note. A special “Live-In” residential-habilitation rate model is designed for homes licensed for three or fewer residents and can be billed up to 365 days — it's built for exactly this small foster-home setting. If you use live-in caregivers, you'll need at least one screened, trained back-up/relief caregiver. Ask APD how the Live-In model fits your plan.

4.2 Path B — open a group home (4–15)

Path B is more work, more money, and more revenue: a group home serves four to fifteen residents and is licensed as a group home facility under 65G-2.0072. The licensing backbone is identical to a foster home — Pre-Service Training, Level 2 screening, APD provider enrollment, the 65G-2 license with inspection and HCBS-settings survey, and the Full AHCA number. What changes is scale.

- **Capacity & license:** 4–15 residents, licensed as a group home facility. APD reassesses your maximum capacity at each licensing, and any capacity increase requires a new fire inspection, a facility survey, and an amended license.
- **Staffing:** you employ direct-care staff on a schedule that meets residents' needs, plus a facility operator/administrator — a real staffing plan, not just the licensee and relief help (see Part 7).
- **Fire code:** larger homes, and residents who can't self-evacuate, drive stiffer fire requirements from the fire authority (see Part 6).
- **Money:** more beds means more revenue, but also more overhead — staff, insurance, a larger property. Many owners open a foster home first, learn the ropes, then open group homes.

Everything else carries straight over from Path A. Because the difference is size, staffing, and fire requirements — not the fundamental process — nothing you learn opening a foster home is wasted when you scale to a group home.

PART 5

Enroll with APD, License the Facility & Get Your Full AHCA Medicaid Number

In short: *This is the chain that unlocks payment, and it's the same for a foster home or a group home: learn the standards and train, pass Level 2 screening, enroll as an APD provider, license the facility under 65G-2 (with the inspection and HCBS-settings survey), and enroll with AHCA as a "Full" Medicaid provider.*

- 1. Learn the standards and complete Pre-Service Training.** Read Rule 65G-2, F.A.C. and the iBudget Waiver Handbook closely — they define your facility standards and your service. Complete APD's required Pre-Service Training before you apply; check APD's site for dates.
- 2. Pass Level 2 background screening.** You and the required household members or staff complete fingerprint-based Level 2 screening (ss. 393.0655, 435.04, 409.907, F.S.). A disqualifying record stops enrollment, and you can't be a suspended or terminated Medicaid/Medicare provider anywhere.
- 3. Enroll as an APD provider.** Complete the APD provider enrollment application for the services you'll deliver (Residential Habilitation), meeting the iBudget Handbook and Florida Administrative Code requirements. Becoming a provider is a free process.
- 4. Apply for your 65G-2 facility license.** Because Residential Habilitation is a licensed service, you submit a separate APD licensing application for the foster or group home. Your regional licensure specialist reviews it, then conducts a facility inspection for physical and safety standards, plus an HCBS-settings survey — both required before the license is issued (see Part 6).
- 5. Get your "Full" Medicaid provider number from AHCA.** Apply to AHCA and enroll as "Full" (the option to bill and receive payment directly from Medicaid). A "limited" enrollment only lets managed-care plans pay you, and the type can't be changed later without starting over — so get this right the first time.

What this means for you: licensing is a quality gate, not a form. APD wants evidence — a secured home, real policies, completed training, a clean screening — that you can safely house and support people. The operators who get through cleanly treat the 65G-2 standards and the iBudget Handbook as the blueprint for the whole operation, so building your home and policies to the standard does double duty: it earns your license AND it's how you'll actually run. Keep the three tracks moving together, and lean on your regional licensure specialist — they'll walk you through the application and the inspection.

PART 6

The Home — What Your House Actually Needs

In short: *Even a small foster home is a licensed facility under 65G-2 — inspected before licensure, fire-inspected annually, monitored monthly by APD, running monthly fire drills. Here's the honest fire-sprinkler answer, the full 65G-2 checklist, and the two housing rules (zoning and room-and-board) that trip people up.*

6.1 An honest heads-up for Florida

Florida is more “facility-oriented” than some states. Even a small foster home for three people is a licensed facility under Rule 65G-2 — it's inspected before licensure, fire-inspected annually, monitored by APD (at least monthly), and it runs monthly fire drills. That's more structure than a plain private-home host arrangement, so plan for real recordkeeping from day one. The upside: the standards are clear and written down, and this guide walks them.

6.2 “Do I need fire sprinklers?” — the honest answer

It depends on your home's size and your residents. Florida residential facilities are inspected for fire safety by the local authority having jurisdiction (or the State Fire Marshal) under Chapter 69A-38, F.A.C. — the Uniform Fire Safety Standards for these homes. For a small foster home with ambulatory residents, full commercial sprinklers usually aren't required; larger group homes, and homes serving residents who can't self-evacuate, are where sprinkler and stricter requirements tend to come in. Because it's set by the fire code and your local fire authority, confirm your specific home with them BEFORE you buy or renovate — this is the single most expensive thing to get wrong.

6.3 The home checklist (Rule 65G-2)

- ☐ Smoke and carbon-monoxide detectors in good working condition (a missing/nonworking detector is a Class II violation).
- ☐ An annual fire-safety inspection by the local fire authority or State Fire Marshal (Chapter 69A-38), plus documented monthly fire drills kept on file for a year.
- ☐ A written emergency and disaster plan; at least one accessible telephone with emergency numbers posted.
- ☐ Safe, sanitary, sound construction — floors, walls, windows, and doors in good repair; safe electrical and utilities.
- ☐ Bedrooms arranged for privacy; a maximum of two residents per bedroom for licenses issued under the current rule.
- ☐ A kitchen equipped to store, prepare, and serve meals for the licensed number of residents.
- ☐ No architectural barriers for residents with physical impairments (accessibility).
- ☐ Distinctive Florida rule: mobile or manufactured homes may NOT be used as a foster home, group home, or residential habilitation center.

6.4 Two housing things that trip people up

- **Zoning — Florida has a statewide protection.** Under section 419.001, F.S., a “community residential home” of six or fewer residents is allowed in single-family and multifamily residential zoning as a matter of right — treated like any family home. Homes for 7–14 residents must notify the local government and generally can't be within 1,000 feet of another such home. So a foster home (≤ 3) or a small group home (≤ 6) is protected residential use; still confirm with your city/county.
- **Room and board vs. the Medicaid payment.** Medicaid pays for the SERVICE (residential habilitation), not the resident's rent and food. Residents pay their own room and board (usually from SSI) under a residency agreement — keep it current and completely separate from what you bill Medicaid.

Do NOT buy or renovate before you confirm the home. *The costliest mistakes in this business are made at the property stage — buying a home that can't pass fire code for your intended size, or a manufactured home that can never be licensed at all, or renovating before the fire authority has told you what your specific home and residents require. Before money changes hands: confirm zoning under s. 419.001 with your city/county, and confirm fire*

requirements (including whether sprinklers apply) with your local fire authority under Chapter 69A-38. A single phone call to each saves five-figure mistakes.

6.5 The HCBS-settings survey — the part people underestimate

Beyond the physical inspection, APD runs an HCBS-settings survey before your license is issued, and it's a different kind of check: instead of asking whether the building is safe, it asks whether the home genuinely functions as the resident's HOME rather than an institution. Federal HCBS rules require that residents have real rights in the setting, and your policies and daily practice have to reflect them. Build the home and your policies to these from the start — they're not optional add-ons, they're a condition of licensure:

- **A real residency agreement:** each resident has a lease or residency agreement giving them the same protections a tenant would have, kept current and separate from your Medicaid billing.
- **Privacy and choice:** residents choose their daily schedule, have privacy in their own space (with a lockable bedroom door where appropriate), can have visitors at any time, and control their own money and personal belongings.
- **Full access to the community:** residents can come and go and take part in community life; the home supports, rather than restricts, that access.
- **Food and daily life on their terms:** residents can access food at any time and take part in choices about meals, activities, and routines — the home runs around them, not the schedule around the home.

Any restriction on these rights has to be individually justified in the person's plan for a specific assessed need, documented, and time-limited — a blanket house rule that limits everyone won't pass the survey. Getting this right up front is far easier than retrofitting a home and a set of policies that were built like an institution.

6.6 Documentation & staying inspection-ready

Whatever your state requires, the agencies that pass surveys and payer audits cleanly all do the same thing: they keep complete, current files from day one instead of scrambling the week before an inspection. Build these habits now, because reconstructing records after the fact is nearly impossible:

- **A file for every caregiver:** the application, background checks, training and competency records, health/TB screening, and a signed acknowledgment of your policies — all dated and kept current.
- **A file for every client:** the assessment, the service/care plan, the signed service agreement, consents, ongoing visit notes, and any payer authorizations.
- **A visit record for every shift:** who worked, when they arrived and left, and what care was provided — and, for Medicaid, the matching EVV record.
- **An incident and complaint log:** every incident, injury, and complaint and exactly how you resolved it — inspectors look for a real, used quality-improvement process, not a binder that's never been opened.

PART 7

Who You Actually Need — Staffing

In short: A foster home (≤ 3) you can often run yourself as the licensee with a screened relief caregiver. A group home (4–15) needs a real team — operator, direct service providers, a residential supervisor, nurse oversight where required, and billing support — though one person can wear several hats at the start.

7.1 The roles

Foster home (≤ 3 residents): you can often run it yourself as the licensee, with at least one screened, trained back-up/relief caregiver (required if you use live-in caregivers). The “Live-In” rate model is designed for exactly this small setting. Group home (4–15 residents): plan for a real team —

- **Operator / administrator:** runs the home and owns compliance and the APD license.
- **Direct service providers (DSPs):** the caregivers who deliver day-to-day support and residential habilitation, scheduled to meet residents' needs around the clock.
- **Residential supervisor:** oversees the DSPs and daily operations in a larger home.
- **Registered Nurse:** for medication and health oversight (required at higher levels of care / Specialized Medical Home Care); often part-time or contracted at first.
- **Billing / administrative support:** submits Medicaid claims in FMMIS and tracks authorizations and payments.

All staff must clear Level 2 background screening and complete required training — including zero-tolerance/abuse-neglect, CPR/First Aid, and medication training where applicable — per the iBudget Handbook and 65G-2. Keep every staffer's screening and training records current and on file, because APD monitors for exactly this.

7.2 Recruiting and keeping DSPs — the real make-or-break

Direct service providers are the heart of a group home and the hardest thing to keep. DSP turnover is high across the whole field, and a home that can't reliably staff its shifts can't safely support residents or keep its license in good standing — so treat recruiting and retention as your core operation, not an afterthought.

- **Recruit continuously and hire for heart:** specific tasks can be trained; patience, reliability, and genuine care for people with disabilities cannot. Check references for dependability.
- **Onboard to the standard:** real orientation, the required Level 2 screening and training (zero-tolerance, CPR/First Aid, medication where applicable), and a competency check before a DSP supports a resident alone — this protects residents and satisfies APD.
- **Retain deliberately:** consistent schedules, correct and on-time pay, real support when a DSP hits a hard day, and recognition. Every DSP who quits costs you re-hiring, re-training, and often continuity for the residents they supported.

PART 8

Policies, Records & Staying Compliant

In short: *Your policies and documentation are what APD licenses and monitors against. Build them to the 65G-2 standard from day one — client rights and grievances, health and safety, incident reporting and abuse/neglect handling, and service delivery.*

Your policies and procedures — client rights and grievances, health and safety, incident reporting and abuse/neglect handling, and service delivery — must be documented. This documentation is both the licensing requirement and the backbone of a safe, well-run operation, and it's exactly what a manual like PolicyReady's Florida group home manual is built for: ready to customize, just add your details. You also keep clear records covering medications, emergencies, fire drills, and the rights of each resident, and each person's support plan drives what you deliver and record. Build a simple system that keeps each resident's plan, service records, drill logs, and incident documentation current, because APD monitors it and can survey it at any time.

8.1 Documentation & staying inspection-ready

Whatever your state requires, the agencies that pass surveys and payer audits cleanly all do the same thing: they keep complete, current files from day one instead of scrambling the week before an inspection. Build these habits now, because reconstructing records after the fact is nearly impossible:

- **A file for every caregiver:** the application, background checks, training and competency records, health/TB screening, and a signed acknowledgment of your policies — all dated and kept current.
- **A file for every client:** the assessment, the service/care plan, the signed service agreement, consents, ongoing visit notes, and any payer authorizations.
- **A visit record for every shift:** who worked, when they arrived and left, and what care was provided — and, for Medicaid, the matching EVV record.
- **An incident and complaint log:** every incident, injury, and complaint and exactly how you resolved it — inspectors look for a real, used quality-improvement process, not a binder that's never been opened.

PART 9

The Person's Journey — Eligibility to Move-In

In short: *You can't get paid until a resident is living with you and residential habilitation is authorized on their cost plan. The path runs from APD eligibility and the waiting list, through a funded waiver slot and the QSI assessment, to the WSC's support plan, the match to your home, and move-in.*

- 1. The person qualifies — and often waits.** To receive iBudget services, the person must have an APD-eligible developmental disability (s. 393.063, F.S.) that began before age 18, meet an ICF/IID level of care, and be Medicaid-eligible. Florida has a large waitlist, so most clients have waited for a funded waiver slot.
- 2. The WSC builds the plan.** The client's Waiver Support Coordinator develops their support plan and cost plan, informed by the QSI assessment, and identifies residential habilitation as a needed service.
- 3. The match.** The WSC and family look for a licensed home that fits the person's needs and preferences — which is where your licensed, HCBS-compliant home comes in. Marketing to WSCs in your area is how homes fill.
- 4. Authorization.** The service and rate are authorized on the client's cost plan for your home — this is what makes your service billable.
- 5. Move-in and sign.** The resident signs a residency agreement (room and board) and moves in. Now you're delivering a billable service.

What this means for you: because families and WSCs choose the home and slots are limited, your census depends on relationships and reputation, not advertising. Get known to your APD regional office and above all to the WSCs in your area as the operator who runs a safe, HCBS-compliant home, communicates, and handles a placement smoothly — and the matches follow.

***Plan your business around the waitlist reality.** Florida's waiting list is long, and funded slots open on their own schedule — which shapes how you should plan. Don't build a model that assumes you'll fill every bed the month you're licensed; assume a gradual build as WSCs learn to trust you and matches come through. Two practical moves help: start with a foster home so your setup and staffing costs are small while you build, and stay in regular, helpful contact with the WSCs in your area so that when one of their clients is choosing a home, you're the name they think of first. Patience plus reliability is the whole strategy — the demand is real, but it arrives one authorized resident at a time.*

PART 10

How You Get Paid — Codes, the Daily Rate & Your First Payment

In short: The core service you bill is Residential Habilitation, paid as a DAILY rate that rises with the resident's Level of Supports. You bill it in FMMIS with HCPCS codes and modifiers. Room and board is separate — never balance-bill the resident for a covered service.

10.1 What you bill for — Residential Habilitation

The core service you bill is Residential Habilitation — supervision and training in daily-living, homemaking, and social skills. It's paid as a daily rate, and the rate depends on the type of residential habilitation and the client's Level of Supports (basic, minimal, moderate, extensive, or intensive). Florida bills these through FMMIS (the state's Medicaid system) using HCPCS codes with modifiers that identify the type.

Type of Residential Habilitation	HCPCS (base)	Billed as
Standard / Behavior-Focused / Intensive-Behavior	T2016 (+ modifiers)	Daily rate
Enhanced Intensive Behavior	T2025 (+ modifiers)	Daily rate (or monthly)
Live-In (homes licensed for 3 or fewer)	T2016 Live-In model	Daily rate, up to 365 days

So the everyday code for most homes is T2016 (Standard, Behavior-Focused, or Intensive-Behavior residential habilitation), billed per day; the most intensive tier uses T2025. Modifiers on the claim pin down the exact type and level.

10.2 How much — and what moves the number

- **Level of Supports:** the daily rate rises with need (basic → intensive). For example, on the 2024 rate table an Intensive-Behavior residential habilitation day ran about \$409, and an Enhanced Intensive Behavior day ran about \$1,124 — Standard rates are lower. Use the current APD/AHCA rate table for your exact figure.
- **Solo vs. agency:** Florida publishes separate rates for a “solo” provider (self-employed/independent, two or fewer employees) and for an agency provider.
- **Geography:** rates differ by area, with Monroe County (the Keys) paid at a higher “geographical” rate.
- **Who sets it:** rates are set by AHCA/APD in the iBudget Waiver rate table and updated periodically (often effective July 1). Always confirm the current number before you budget.

10.3 The money math — how a home pencils out

Residential-habilitation economics are different from an hourly private-pay business — your revenue is a daily rate per resident, and your biggest cost is staff labor. Lay it out with your own numbers:

- **Revenue:** the authorized daily rate for residential habilitation, per resident, per day — driven by each resident's type and Level of Supports, and billed in FMMIS. More licensed, filled beds at the right level is more revenue.
- **Cost:** staff wages and payroll taxes are the bulk in a group home; then RN oversight where required, administration, insurance, and the property. Most are fixed against your census.
- **The lever:** because the rate is set by the waiver, your margin comes from keeping beds filled with authorized residents and staffing efficiently to their needs — utilization, not rate, is the number you manage. A foster home's Live-In model keeps staffing cost low at the small scale.
- **Room and board stays separate:** the resident's rent and food come from their own income (usually SSI), NOT Medicaid — keep that money and those agreements entirely separate from your service billing, and never balance-bill for a covered service.

10.4 Your first payment

1. **Confirm the authorization on the client's cost plan.** The residential habilitation type, level, and rate for your home.

2. **Deliver and document the service each day.** Your daily documentation is what supports the claim and what APD monitors.
3. **Submit the claim in FMMIS.** The right HCPCS code and modifiers for the type and level, billed per day.
4. **Get paid — and never balance-bill.** Medicaid pays the service; the resident owes only their agreed room and board. Never bill the resident or family for a covered service.

10.5 The billing mistakes that cost Florida homes money

Most payment problems in this business aren't about the rate — they're about clean claims and clean documentation. A handful of avoidable mistakes account for the bulk of denied claims, delayed payments, and paybacks after a monitoring visit. Know them going in:

- **Wrong or missing modifiers:** the base HCPCS code (T2016 or T2025) is only half the claim — the modifiers pin down the type of residential habilitation and the Level of Supports. A missing or wrong modifier is one of the most common denials, so match the modifier to what's authorized on the cost plan.
- **Billing beyond the authorization:** you can only bill what the resident's cost plan authorizes — the type, the level, and the days. Billing a higher level than authorized, or days that aren't approved, triggers a denial or a payback. Confirm the authorization before you bill, and watch for plan changes.
- **Documentation that doesn't match the claim:** every billed day needs daily service documentation behind it. If APD monitors and the records don't support the days billed, the money comes back — so the daily note is not busywork, it's what protects the payment.
- **Choosing “limited” instead of “Full” at AHCA:** the enrollment mistake that stops payment before you even start — a limited enrollment can't be paid directly by Medicaid for iBudget, and fixing it means starting the enrollment over. Confirm you enrolled as “Full.”
- **Letting an enrollment or license lapse:** a lapsed 65G-2 license, an expired provider enrollment, or an out-of-date AHCA record stops payment even while the care continues. Calendar every renewal.

PART 11

What It Costs, How Long It Takes & Getting Placements

In short: A foster home is faster and cheaper to stand up than a group home; either way plan for several months through enrollment, licensing, and the AHCA number. Placements come through the WSCs, so those relationships are your census.

11.1 Timeline & startup costs

Plan for a multi-step process: Pre-Service Training, background screening, provider enrollment, the 65G-2 facility license (with the inspection and HCBS-settings survey), and your AHCA Medicaid number. Realistically this takes several months end to end, and you must have the home secured before you apply. A foster home is generally faster and cheaper to stand up than a group home. Confirm current timelines with your APD regional office.

Becoming an APD provider is free, but plan for: business formation and insurance; securing and preparing the home (fire-safety items, smoke/CO detectors, minor fixes to pass inspection); Pre-Service and required staff training; background screening fees; and your policies and procedures. A group home adds staffing payroll and a larger property. Because Medicaid pays weeks after you deliver — and after a resident is even placed — carry a working-capital cushion through the ramp. Confirm current fees with APD and AHCA.

11.2 Getting your first placements

Placements don't come from advertising — they come through the WSC process and your reputation. Here's a simple plan:

- 1. Get known to your APD regional office.** Call APD (1-866-273-2273) and your regional office; ask who your licensure specialist is and what the current need for homes is in your area.
- 2. Build relationships with WSCs.** They write the support plans and match clients to homes, and families choose. Be the operator they trust to run a safe, compliant home, communicate, and handle a placement smoothly.
- 3. Be responsive and reliable.** Answer the phone, say yes to a good match, handle move-in smoothly, and follow through — one good placement earns the next referral.

PART 12

Who to Call & Exactly What to Ask — Every Verify, in One Place

In short: APD and AHCA are the center of gravity, with your local zoning, building, and fire offices close behind. Statewide contacts are the same for everyone; the local calls are yours. Here's the list, the scripts, and Miami and Tampa worked all the way out.

12.1 The statewide offices (same for everyone)

For...	Contact
Becoming a provider & licensing (APD)	APD — 1-866-273-2273 (1-866-APD-CARES) · apd.myflorida.com → Providers
Your Medicaid provider number	AHCA (Florida Medicaid) — ahca.myflorida.com → Medicaid provider enrollment
iBudget rates & billing	APD → Provider Billing, Payments & Rates (rate tables); billed in FMMIS
Fire-safety standards	Local fire authority / State Fire Marshal — Chapter 69A-38, F.A.C.
Find your APD regional office	apd.myflorida.com/region
Business, EIN & NPI	sunbiz.org · irs.gov (EIN) · nppes.cms.hhs.gov (NPI)

12.2 The local calls — and exactly what to ask

① Your APD regional office — your most important call

"I want to open a foster home / group home in this county. Who is my licensure specialist, and can you walk me through the application and inspection?"

"What's the current need for homes in this area, and are there WSCs looking for placements?"

② City or county planning / zoning — "can I legally run a home here?"

"I want to open a community residential home for adults with developmental disabilities — [three / up to six] residents. Under section 419.001, F.S., is this allowed at [address] as a single-family residential use?"

"For a home of 7-14 residents, what notice and the 1,000-foot spacing rule apply here?"

"Any local permits or business tax receipt I need? Can you confirm in writing?"

③ Local fire authority / State Fire Marshal — fire code

"I'm licensing a foster/group home under APD. What does Chapter 69A-38 require for my home's size and residents — alarms, extinguishers, exits, and do I need sprinklers?"

"Who performs my annual fire inspection, and how do I schedule the initial one?"

12.3 Fully worked examples — Florida's two biggest metros

Most people start in or near one of these two, so each one's real call list is done for you below. If you're elsewhere, use these as a template — APD provider enrollment and AHCA are statewide; the local piece is mainly zoning, building, and fire.

- Miami (Miami-Dade County):** APD Southern Region — 401 NW 2nd Avenue, Suite 811, Miami, FL 33128; (305) 349-1478 (serves Miami-Dade and Monroe). Zoning: City of Miami / Miami-Dade County zoning — confirm your address and cite s. 419.001, F.S. for a community residential home of six or fewer. Building & fire: Miami-Dade (or your city's) building department for permits; Miami-Dade Fire Rescue (or the State Fire Marshal) for the Chapter 69A-38 inspection. The local quirk: Monroe County (the Keys) is in this same APD region and is paid at Florida's higher "geographical" Medicaid rate — worth knowing if you're down in the Keys.
- Tampa (Hillsborough County):** APD SunCoast Region — 1313 North Tampa Street, Suite 515, Tampa, FL 33602; (800) 615-8720; SunCoast Provider Development: (813) 233-4325 (serves Hillsborough, Pinellas, Pasco, Manatee, Sarasota and more). Zoning: City of Tampa / Hillsborough County zoning — the same s. 419.001 protection applies statewide. Building & fire: City of Tampa (or Hillsborough County) building department for permits; the local fire authority for the annual Chapter 69A-38 inspection. The local quirk: confirm whether

your address is inside the City of Tampa or unincorporated Hillsborough County — the permitting and fire offices differ.

12.4 How to find your own city or county's numbers

1. **Start with your APD regional office.** They know the local landscape and your licensure specialist — apd.myflorida.com/region.
2. **For zoning, go local.** Search “[your city] FL zoning” or “[your county] FL planning,” or call your city/county main line and ask for Planning/Zoning; mention section 419.001 community residential homes.
3. **Not sure who covers you?** Call 2-1-1 Florida and ask.

Common questions, answered

Do I have to take Medicaid?

No. Many successful agencies run entirely on private pay, long-term-care insurance, and veterans' benefits — it's often the smarter way to start, because you earn immediately without the enrollment, contracting, and EVV that Medicaid requires. You can add Medicaid later once you have cash flow and a team.

Can I run this from home at first?

Often yes for a small non-medical agency — the care happens in the client's home, so you may not need a storefront to start. Confirm your local zoning and your license/registration rules (some require a business address), and know you'll want real office space as you grow.

Should my caregivers be employees or independent contractors?

Almost always W-2 employees. Classifying caregivers as 1099 contractors to save on taxes is one of the most common — and most expensive — mistakes in home care; it invites wage-and-hour and tax liability, and most payers and insurers require employees. Treat them as employees and carry workers' comp.

How much cash do I really need to start?

Enough to cover several weeks — ideally a couple of months — of caregiver payroll before your first payer reimbursements arrive, plus your license/insurance/software setup. Under-capitalization is the number-one reason new agencies fail; line up a business line of credit before you scale.

How long until it's profitable?

Most agencies take several months to a year to build a steady client base and referral flow. Starting on private pay shortens the runway because you're paid quickly; Medicaid volume comes later and pays slower. Plan your finances for a ramp, not an overnight full census.

Do I need experience in health care?

It helps, but plenty of successful owners come from business, sales, or family-caregiving backgrounds and hire the clinical expertise they need (for example, a nurse where the state requires one). What matters more is being organized, responsive, good with people, and disciplined about compliance and cash.

What actually makes one agency win over another?

Reliability. Families and referral sources choose — and stay with — the agency that answers the phone, starts care fast, sends the same dependable caregiver, communicates, and never leaves a shift unstaffed. Get that right and referrals compound; get it wrong and no amount of marketing saves you.

Do I have to open a big group home, or can I start smaller?

You can — and probably should — start smaller. A licensed foster home for up to three residents is the fastest, cheapest way in, with fewer fire requirements and a lighter staffing plan, and you can often run it yourself with a screened relief caregiver. The licensing backbone is identical to a group home, so it's the best way to learn APD's system before you scale up. Many strong operators grow from a foster home into group homes.

Why do I need both APD and AHCA?

APD runs the iBudget waiver — it enrolls you as a provider, licenses your home under 65G-2, and monitors you. AHCA is Florida's Medicaid agency and issues the provider number that lets you actually be paid. You need APD enrollment and a 65G-2 license to deliver the service, and a “Full” AHCA Medicaid number to bill and receive payment directly — so you generally need both, plus a WSC authorizing the service on a resident's cost plan.

Does the waiver pay for the person's rent and food?

No — Medicaid pays for the SERVICE (residential habilitation), not room and board. The resident's rent and food come from their own income, usually SSI, under a separate residency agreement. Keep the two completely separate in your books and agreements, and never balance-bill a resident for a covered service — mixing them is a common and serious mistake.

Can I use a mobile or manufactured home?

No. Florida rule specifically prohibits using a mobile or manufactured home as a foster home, group home, or residential habilitation center. Confirm any property's eligibility — and its fire and zoning status — before you buy or renovate.

Do I have to choose “Full” enrollment with AHCA?

Yes, to be paid by APD. “Full” lets you bill and receive payment directly from Medicaid; a “limited” enrollment only lets managed-care plans pay you, and the enrollment type can't be changed later without starting the enrollment over. It's one of the easiest steps to get wrong, so choose “Full.”

You can do this

Opening a group home business in Florida is real work — it's a licensed facility, so there's more structure than a casual host arrangement — but it's a real, walkable path, and people across the state do it every day, giving someone a real home in the community. You don't have to be a lawyer or hire a consultant. Whether you start small with a foster home or open a full group home, take it one step at a time: set up the business, learn the 65G-2 standards, complete Pre-Service Training and screening, enroll as an APD provider, license the facility, get your Full AHCA Medicaid number, ready the home, and build the WSC relationships that bring you placements. Verify the current details with the State as you go, and lean on your APD regional office and your local fire authority — that's what they're there for.

Reminder. Prepared for PolicyReady.org and grounded in Florida APD and AHCA rules, Chapter 393, F.S., Rule 65G-2, F.A.C., Chapter 69A-38, F.A.C., section 419.001, F.S., and the iBudget Waiver Handbook and rate tables as of 2026. This is general information, not legal, tax, or billing advice. Requirements, rates, and codes change — always verify current details with the State of Florida before you act.